



Health migrations: How pre-migration healthcare experiences and US healthcare challenges shape immigrants' transnational healthcare practices

Tiffany D. Joseph ^{*} , Vibhustuti Thapa 

Department of Sociology and Anthropology, Northeastern University, United States

ARTICLE INFO

Keywords:

Immigrant health care
US healthcare system
Transnationalism
Language concordance
Cultural discordance

ABSTRACT

Research examining immigrants' healthcare experiences in host countries have found that various structural and socio-cultural challenges impede their healthcare access. In response, migrants address unmet medical needs by engaging in what we call "transnational healthcare practices" (THCPs). But less research has explored how migrants' pre-migration healthcare experiences influence their negotiation of care across transnational contexts. This paper aims to fill that gap by addressing four questions: 1) How do pre-migration healthcare experiences at home shape immigrants' orientation to transnational healthcare practices (THCPs)?; 2) What structural and socio-cultural healthcare challenges facilitate migrants' THCPs?; 3) What THCPs do immigrants engage in?; and 4) How do providers' views of THCPs differ from migrants' perceptions? Data comes from 132 interviews conducted with Latin American immigrants and healthcare providers in Boston from 2012 to 2019. Findings indicate that: 1) pre-migration healthcare experiences generate "here" and "there" comparisons that facilitate migrants' THCPs in the US; 2) the complex and costly US healthcare system poses challenges that push migrants' THCP engagement; 3) migrants engage in THCPs like receiving medication from home, having transnational telehealth consultations, and traveling home for care; and 4) providers caution against THCPs because of health consequences, although migrants view them positively. Ultimately, THCPs represent migrants' agency to meet healthcare needs amid structural and socio-cultural challenges in the US healthcare system. Through highlighting the role of pre-migration healthcare practices at home as central to migrants' THCPs, this article contributes to scholarship on transnationalism, immigration, and health care.

Immigration remains a contentious issue in Global North countries as people have moved from Global South nations seeking economic opportunity and escaping conflict, persecution, unstable governments, and climate change impacts. Amid aging populations and economies recovering from the COVID-19 pandemic, some Global North countries' social safety nets have become more restrictive, particularly towards immigrants (LaFleur and Mescoli, 2018; Verschueren, 2020). This has also happened in the United States, with noncitizens of various statuses being excluded from health coverage and access (Joseph, 2025; Park et al., 2024).

Furthermore, the US healthcare system is notoriously complicated to navigate, with race, social class, and legal status exacerbating certain individuals' challenges accessing care (Hoffman, 2025; Joseph, 2025; Park et al., 2024). Immigrants, especially those of color, are more likely to have negative healthcare experiences (Joseph, 2025; Van Natta, 2023). To mitigate those experiences, some immigrants negotiate their health care across borders, engaging in what we call "transnational

healthcare practices" (THCPs). THCPs are practices immigrants engage in to receive health care, or to address health problems, by relying on social networks, cultural remedies, or healthcare providers in their home countries while living in the host country. In this paper, we address four questions regarding THCPs: 1) How does pre-migration healthcare system exposure shape immigrants' orientation to THCPs?; 2) What structural and socio-cultural healthcare challenges facilitate migrants' THCPs?; 3) What THCPs do immigrants engage in?; and 4) How do providers' views of THCPs differ from migrants' perceptions?

Various studies have examined how people negotiate health care across borders (Levitt et al., 2023; Torres and Waldinger, 2015). However, this study is unique because it explores the intricacies of THCPs as they relate to immigrants' exposure to healthcare systems in their home countries before migrating to the US. Specifically, this article utilizes interview data conducted with 132 immigrants and healthcare providers in Boston from 2012 to 2019 after implementation of the Massachusetts and Affordable Care Act (ACA) health reforms. Our concept of THCPs

^{*} Corresponding author. 201 Renaissance Park, 360 Huntington Avenue, Boston, MA, 02115, United States.

E-mail address: t.joseph@northeastern.edu (T.D. Joseph).

<https://doi.org/10.1016/j.socscimed.2026.119382>

Received 25 August 2025; Received in revised form 27 April 2026; Accepted 7 May 2026

Available online 8 May 2026

0277-9536/© 2026 Elsevier Ltd. All rights are reserved, including those for text and data mining, AI training, and similar technologies.

also differs from previously explored concepts of medical tourism – traveling to other countries for medical care – as it reveals the various constraints immigrants experience navigating their care transnationally (Bustamante, 2023; Connell, 2016). Most importantly, we show how pre-migration socioeconomic status and healthcare system experiences shape immigrants' THCPs. This is an important theoretical and empirical contribution to existing scholarship as it reveals continuity in ongoing healthcare strategies rooted in experiences before migration. We begin by discussing relevant transnationalism scholarship and its connection to immigrants' health care. Next, we outline the data used to address our research questions before discussing our findings about how immigrants negotiate their health care “here” and “there.”

1. Literature review

1.1. Transnationalism, migrant health, and care

The concept of transnationalism emerged from an increased focus on immigrant incorporation into destination countries. Specifically, migration scholars examined migrants' social, economic, cultural, and political embeddedness in both home and destination societies and its impacts on their identities (Faist, 2000; Levitt et al., 2023; Portes et al., 1999; Vertovec, 2001). Research on transnationalism and health has since highlighted the role of formal and informal transnational social networks in shaping migrants' healthcare access and experiences (Baldassar, 2014; Bryceson, 2019; Torres and Casey, 2017).

Findings on the physical and mental health implications of transnational social ties are mixed. Studies indicate that transnational connections are associated with better self-rated overall health and improved access to vital information, e.g., for pregnant and postpartum women (McFadden et al., 2014; Chakrabarti, 2010). Other studies show negative associations, like increased distress and poorer health status (Torres et al., 2016; Vermot, 2015). While transnational networks have been found to provide emotional support and community for migrants experiencing isolation and stress, transnational ties have also been linked to outcomes like depression (Bhattacharya, 2011; Sanon et al., 2016; Tiilikainen and Koehn, 2011; Viruell-Fuentes and Schulz, 2009).

Transnational networks thus play a crucial role for migrants seeking health care in destination countries. When migrants experience exclusion from formal healthcare systems, they often rely on markets and transnational relationships with family, community, and non-governmental organizations to meet their needs. Levitt et al. (2023) refer to this network under their “hybrid transnational social protection” (HTSP) framework. Within HTSP, migrants draw on different resource environments, to create “toolkits of care” (Levitt et al., 2023). In addition to maintaining cross-border networks, these toolkits allow them to access allopathic and traditional medicine across countries, like receiving medicine in the mail from the home country or connecting with telehealth providers transnationally (Castañeda and Melo, 2014; Levitt et al., 2023; Menjivar, 2002).

The ability to assemble such toolkits, however, varies and depends on levels of healthcare disenfranchisement, or the inability to exercise formal rights to health care (Ormond and Lunt, 2020). Migrants who face healthcare disenfranchisement but have formal legal status and financial resources may travel internationally for primary, specialty, and non-elective care more freely than counterparts with more precarious status who face limited or riskier care options (Grineski, 2011; Torres and Waldinger, 2015). Nevertheless, studies show that migrants mobilize different forms of transnational “cultural health capital” (CHC), cultural capital leveraged in healthcare contexts to optimize care across distinct transnational institutional arrangements (Shim, 2010). Transnational CHC may include knowledge of different healthcare systems, proficiency in multiple languages, or sensitivity to diverse interactional styles (Amaro et al., 2024; Madden, 2015; Sanon et al., 2016; Shim, 2010). Migrants combine these diverse practices, shaped by intersections of their gender, race, ethnicity, class, and transnational

engagement, to navigate systems to meet their own and their families' healthcare needs (Lafleur and Romero, 2018; Madden, 2015).

1.2. Barriers encouraging transnational healthcare practices (THCPs)

Structural exclusion, discrimination, and inequality in formal healthcare systems in destination countries influence migrants' THCPs (Bustamante, 2019; Horton and Cole, 2011; Joseph, 2025; Levitt et al., 2023). Legal status specifically shapes insurance coverage and healthcare access in many migrants' home countries like the US and beyond, determining eligibility for coverage (Ahmadinia et al., 2022; Joseph, 2025; Van Natta, 2023). Legal status concerns also perpetuate fear of using health services amid heightened immigration enforcement in the Global North (Castañeda and Melo, 2014; Bustamante, 2019, 2023). Cost compounds these barriers (Grineski, 2011; Bustamante, 2019, 2023; Virani et al., 2020). The high cost of US health coverage and services but lower costs of treatments in many migrants' home countries act as “pull factors” for care there (Joseph, 2025; Bustamante, 2023). Cumulatively, these factors generate disenfranchisement, steering people towards transnational healthcare-seeking behaviors (Ormond and Lunt, 2020; Park et al., 2024; Bustamante, 2019, 2023).

Socio-culturally, distrust in providers' motives, like perceptions that US doctors are profit-motivated and only prescribe medications instead of alternative treatments, can also shape THCPs (Horton and Cole, 2011; Levitt et al., 2023; Menjivar, 2002). While some studies suggest that migrants perceive transnational health-related advice as more trustworthy, others find that migrants may be hesitant to trust their home countries' healthcare systems and bureaucracies (Escandell and Tapias, 2010; Smith et al., 2020). Differences in the practices of medicine, patient-provider dynamics, and variations across healthcare systems that shape access drive migrants' reliance on THCPs (Joseph, 2025; Sun and Lin, 2025).

Research on cross-border travel at the US-Mexico border emphasizes how policy contexts and shifts influence THCPs. Quantitative research on US-Mexico travel shows that the ACA extended healthcare access to citizens and certain non-citizens but limited options for those with irregular status (Bustamante, 2019). Leveraging their transnational cultural health capital, migrants across the US-Mexico border and border cities strategized to select different healthcare markets for optimal care (Amaro et al., 2024; Madden, 2015; Bustamante, 2019). In this post-ACA context, and amid increasingly restrictive immigration policies, there remains a need to comprehensively identify and trace factors shaping how individuals strategically navigate different and fragmented systems of care across borders. This article starts filling that gap by qualitatively assessing THCPs among Brazilian, Dominican, and Salvadoran immigrants in a locale much further from the US-Mexico border. Our concept of THCPs generates insights into the implications of migrants' cross-border and transnational connections for their health and health care.

2. Data and methods

Data for this article comes from a larger study conducted by the first author that examined the influence of state and national-level health policies on Latin Americans' healthcare access in Boston, Massachusetts. Boston was an ideal study site because immigrants comprise 20% of the population and it is the capital of Massachusetts, the first state to implement the health reform that became the model for the 2010 ACA. The first author interviewed a total sample of 209 individuals consisting of Brazilian, Dominican, and Salvadoran immigrants; healthcare professionals that provide care to immigrants; and employees from immigrant and health advocacy organizations from 2012 to 2019. This article only includes data from the 132 interviews with immigrants and providers (Table 1). Interviews were conducted at three time periods coinciding with health policy shifts: (1) 2012–2013 after implementation of the Massachusetts health reform, (2) 2015–2016 after

Table 1

Immigrants and providers interviewed in 2012–2013, 2015–2016, and 2019 (total N = 132).

Stakeholder Group	Pre-ACA 2012–2013	Post-ACA 2015–2016 ^a	Post-2016 Election: 2019 ^b
Immigrants	N = 31	N = 39	N = 12
- Brazilians	21	15	8
- Dominicans	10	14	2
- Salvadorans	N/A	10	2
Healthcare Providers at Boston Health Coalition	N = 19	N = 19	N = 12
- Physicians	5	6	5
- Medical Interpreters	4	4	2
- Other Medical Staff	10	9	5
Total	50	58	24

Note.

^a Salvadorans added to the 2015–2016 sample amid political debates about TPS status renewal.

^b 2019 Immigrant Sample is smaller due to recruitment difficulty in socio-political climate.

implementation of the ACA in 2014, and (3) 2019 under the first Trump Administration, which attempted to repeal the ACA and implemented immigration policies that chilled immigrants' health service use. This study design allowed for exploration of complex intersections among subnational and federal policies, alongside health and immigration policies, that perpetuate exclusion and inequality in the healthcare system.

Immigrants described the unique challenges they faced obtaining health coverage and navigating the healthcare system. Brazilians, Dominicans, and Salvadorans are among the largest immigrant groups in Boston. Brazilians are typically undocumented, from middle-income backgrounds, and racialized as White. Dominicans are usually legal

permanent residents (green card holders), from lower-income backgrounds, and racialized as Black. Salvadorans are eligible for temporary protected status, which provides certain public benefits, from lower-income backgrounds, and racialized as Brown. Despite their different national origins, languages, and cultures, alongside their varied physical appearance and documentation statuses, they are racialized as "Latino/a" in the United States. Exploring these groups' healthcare experiences provides a fuller understanding of how Latino/as' heterogeneity influences their health care. Table 2 includes demographic information about the immigrant sample. The first author focused on Brazilians and Dominicans in 2012 but added Salvadorans in 2015 after the influx of Central Americans to the US-Mexico border, some of whom settled in Boston.

Healthcare professionals from the Boston Health Coalition (BHC) yielded insights into how shifting policies influenced their ability to provide care for immigrant patients. BHC is a pseudonym for the hospital network, which has a reputation for providing quality care to Boston area underserved populations. Brazilian immigrants, along with providers who serve the Brazilian community, comprised a larger portion of the sample because they were more willing to participate in the study.

To recruit respondents, the first author posted multilingual fliers in Brazilian, Dominican, and Salvadoran neighborhoods and community centers and attended and volunteered at relevant immigrant advocacy organizations. The first author also used purposive snowball sampling, commonly used for hard-to-reach populations, to recruit immigrant respondents and the providers and advocates who worked with them (Ellard-Gray et al., 2015; Naderifar et al., 2017). All the immigrant respondents were new each year of the study amid IRB concerns about the safety of immigrant respondents. Eleven providers were reinterviewed throughout the study to address their perceptions of policy change impact over time. The first author also interviewed respondents across the different stakeholder groups until there was sufficient overlap in

Table 2

Demographic characteristics of immigrants interviewed in 2012–2013, 2015–2016, and 2019 (N = 82).

Demographics	2012–2013 Immigrant Sample (N = 31)		2015–2016 Immigrant Sample (N = 39)			2019 Immigrant Sample (N = 12)		
	Brazilians (N = 21)	Dominicans (N = 10)	Brazilians (N = 15)	Dominicans (N = 14)	Salvadorans (N = 10)	Brazilians (N = 8)	Dominicans (N = 2)	Salvadorans (N = 2)
Gender (#women)	12	5	8	10	6	5	2	2
Median Age (years)	40	55	43	56	40	46	55	34
Average Time in US (years)	12	14	10	21	19	11.5	28	6
Average Individual Monthly Income	\$3969	\$480	\$1720	\$843	\$1383	\$3017	\$3050	\$1380
Employed at Time of Interview	16	6	11	8	8	6	2	2
Customary Occupations	Hairstylists, Childcare, Cleaning, Education	Childcare, Cleaning	Childcare, Cleaning, Landscaping	Childcare, Restaurants	Childcare, Clerical, Restaurants	Cleaning, Nurse Assistant	Dentist, Housecleaning	Clerical
Documentation Status (at interview)								
Undocumented	6	3	6		6	5		1
DACA						1		
Tourist Visa (B1)	2		1					
Religious Visa (R1)	1							
Student Visa (F1)			3					
Work Visa (H1A/B)	1		1					
Temporary Protected Status (TPS)	N/A	N/A	N/A	N/A	1			
Green Card/LPR	10	4	3	11	2			1
Naturalized Citizen	1	3	1	3	1	2	2	
Health Coverage Type								
Uninsured (N)	1		2	2	3	1		
Public (N)	12	8	10	10	7	3	2	2
Private (N)	8	2	3	2		4	2	

participants' responses regarding immigrants' healthcare experiences each year of the study. This aligns with what qualitative scholars refer to as "saturation" in line with the grounded theory approach (Creswell, 2007; Harding, 2013; Strauss and Corbin, 1998). The one exception was in 2019 where the immigrant sample was smaller amid recruitment difficulties due to the first Trump administration's draconian immigration policies.

This recruitment strategy was effective given the intensifying racialized anti-immigrant sociopolitical climate during the study. Because this qualitative study did not have a large random sample, the findings are not generalizable to broader populations. Nevertheless, this study design produced new insights into difficult-to-observe challenges that marginalized groups experience in the healthcare system. Such challenges may be present in other empirical contexts (Becker, 1998).

Interviews with immigrants were conducted in Brazilian Portuguese or Spanish and lasted 60 min. These interviews assessed immigrants': (1) demographic background in the US and home country; (2) access to and experiences with health coverage and healthcare system in the US and home country; (4) perceptions and experiences of discrimination in the US and home country; (5) racial and skin tone classifications in the US and home country and (6) awareness of health policy and socio-political shifts in the US. Interviews with BHC providers lasted 60 min and examined: (1) types of health services provided; (2) demographics and health coverage of patients; (3) impact of racial, language, and cultural differences on patient-provider interactions; (4) common physical and mental health issues and challenges among immigrant patients; (5) perceptions of discrimination among immigrants; and (6) role of policy changes in provision of care for immigrants.

All but two of the 132 immigrant and provider interviews were audio recorded. Two immigrants declined recording amid fears about what might happen to the recording. After conducting each interview, the first author wrote fieldnotes summarizing major themes, interesting comments, and body language. A professional transcription company transcribed the recorded 130 interviews. The authors reviewed each transcript while listening to the audio recording to ensure accuracy between the audio recording and transcript. Then the authors formatted the revised transcripts for data analysis in NVivo qualitative software.

To analyze the findings, the authors used an inductive, grounded theory approach, which is standard in qualitative research to develop theoretical categories from systematic data analysis (Creswell, 2007; Harding, 2013; Strauss and Corbin, 1998). The authors used open and focused coding, reading each transcript closely and developing an extensive list of recurring themes, with three-word phrases describing various aspects of immigrants' healthcare access related to health coverage and experiences with healthcare systems in the US and their home countries before and since migrating to the US (Strauss and Corbin, 1998). It was from this coding process that the authors created transnational healthcare specific codes: 1) transnational ties; 2) health coverage in the US and at home; 3) healthcare and provider experiences in the US and at home; and 4) types of medical help in the US and at home. The authors placed related interview portions under these codes and created sub-codes corresponding to each stakeholder group to compare the immigrant and provider groups. Each interview was analyzed in the language of the interview, which reduced losing translation nuances before and during analysis. Only the quotes that appear in this paper were translated. This intensive process continued until all interview transcripts were analyzed and led to the findings presented.

2.1. Findings

Interview data reveal that immigrants engaged in various transnational healthcare practices (THCPs) to meet their healthcare needs. Importantly, differences in pre-migration socioeconomic status and healthcare exposure shaped those THCPs. The findings are organized to align with the paper's research questions: 1) How does pre-migration healthcare system exposure shape immigrants' orientation to THCPs?

2) What structural and socio-cultural healthcare challenges facilitate migrants' THCPs?; 3) What THCPs do immigrants engage in?; and 4) How do providers' views of THCPs differ from migrants' perceptions? We address each question in the sections below.

2.2. Pre-migration healthcare system exposure and immigrants' THCP orientation

Immigrants encountered the US healthcare system with a "transnational health frame of mind," which we define as a mindset whereby individuals address health concerns drawing from experiences with, insights from, or access to healthcare services, treatments, or remedies from two or more countries where they have citizenship, ethnic, or cultural ties. This frame of mind is shaped by their pre-migration exposure to the healthcare system in their home countries. Immigrants' pre-migration socioeconomic status (SES) influenced whether they had health insurance, the type of insurance they had (private or public), and their access to a regular provider. Each year of the study, Brazilians typically came from middle SES backgrounds, which afforded them access to private health coverage and regular medical providers before migrating (Table 3). While some Dominicans were from middle SES backgrounds with access to private coverage and regular providers pre-migration, most Dominicans and Salvadorans were from lower SES backgrounds without similar access. This varied depending on the year of the study. Such SES differences facilitated different levels of health literacy at home that translated into the US context after arrival. Table 3 includes data on immigrants' pre-migration education level, health coverage, type of coverage, and access to a regular provider.

In 2012–2013, a higher proportion of Brazilians reported having access to health coverage, usually private, and a regular provider at home compared to Dominicans. But in 2015–2016, about half of both Brazilians and Dominicans reported having private health coverage, with slightly more Dominicans having access to a regular provider than Brazilians. Though the small 2019 sample makes inter-group comparisons implausible, most Brazilians, one Dominican, and one Salvadoran reported having access to private coverage and regular providers before migrating.

Across each year of the study, immigrants with pre-migration coverage – usually private – reported mostly positive experiences obtaining care in their home countries. For example, Juan, a Dominican immigrant interviewed in 2016 with private coverage and a regular provider before migrating, reported that providers seemed more personally concerned about patients and there was less bureaucracy to get necessary care in the Dominican Republic (DR): "I believe there [in DR] is more personal attention, a better relationship with doctors there. Here, not so much. And here, if you have an emergency, it takes much longer to resolve."

Similarly, a Brazilian immigrant named Adelia interviewed in 2016, who had private coverage and a regular provider before migration, felt it was easier to see a specialty doctor in Brazil because of less insurance bureaucracy: "In Brazil, you go directly to the doctor you need to see. If that doctor thinks you need to see a different doctor, they will make it happen. It takes much less time ... The system is not as bureaucratic."

A Salvadoran immigrant named Magdalena interviewed in 2019, who had public coverage and a regular provider before migrating, highlighted the more personal patient-provider relationship, noting that doctors gave their personal contact information to patients, generating more trust and accessibility:

I feel the difference between doctors from El Salvador and those from the US. There, they relate more to the patient, to the extent that the patient has the doctor's personal phone number. Here ... I can't directly communicate with the doctors.

Such positive experiences in migrants' home healthcare systems created a transnational health frame of mind, shaping perceptions of the US healthcare system. Pre-migration healthcare exposure created

Table 3

Pre-migration education and healthcare exposure of immigrants interviewed in 2012–2013, 2015–2016, and 2019 (N = 82).

Pre-migration Education and healthcare exposure	2012–2013 Immigrant Sample (N = 31)		2015–2016 Immigrant Sample (N = 39)			2019 Immigrant Sample (N = 12)		
	Brazilians (N = 21)	Dominicans (N = 10)	Brazilians (N = 15)	Dominicans (N = 14)	Salvadorans (N = 10)	Brazilians (N = 8)	Dominicans (N = 2)	Salvadorans (N = 2)
Education Completed								
Primary	2	6	3	4	4	0	0	0
Secondary	5	1	3	3	3	1	0	0
Some College	4	1	3	1	0	1	0	0
Graduated College	5	1	3	6	2	6	1	2
Post-Graduate Degree	5	0	3	0	0	0	1	0
Not Reported	0	1	0	0	1	0	0	0
Health Coverage	Yes: 13 No: 8	Yes: 2 No: 7	Yes: 7 No: 7	Yes: 7 No: 7	Yes: 3 No: 6	Yes: 4 No: 3	Yes: 1 No: 1	Yes: 2
Coverage Type	Private: 12 Public: 1 N/A: 8	NR: 1 Private: 1 Public: 1 N/A: 7 NR: 1	DNR: 1 Private: 7 Public: 0 N/A: 8	Private: 6 Public: 1 N/A: 7	DNR: 1 Private: 0 Public: 2 N/A: 6 DNR: 2	DNR: 1 Private: 3 Public: 0 N/A: 3 DNR: 2	Private: 1 Public: 0 N/A: 1	Private: 1 Public: 1
Regular Care Provider	Yes: 12 No: 8 NR: 1	Yes: 2 No: 7 NR: 1	Yes: 7 No: 7 DNR: 1	Yes: 9 No: 5	Yes: 0 No: 3 N/A: 6 DNR: 1	Yes: 4 No: 3 DNR: 1	Yes: 1 No: 1	Yes: 2

Note: N/A = Not Applicable; NR = Not Reported; DNR = Did Not Remember.

different starting points for immigrants' navigation of the US healthcare system, yielding robust comparisons between health care in the US and at home. When migrants experienced healthcare access challenges in the US, they were reminded of positive experiences at home, which facilitated their likelihood to engage in THCPs. But challenges in the US healthcare system would also play a role in migrants' THCP engagement.

2.3. US healthcare challenges as a driver of THCPs

Migrants reported experiencing a range of healthcare challenges that likely played a role in their decision to engage in transnational healthcare practices in the US. Those challenges were both structural—related to how the US healthcare system functions—and socio-cultural—related to social and cultural differences in the practice of medicine between their home and US healthcare systems. We first discuss the structural and socio-cultural challenges noted among migrant respondents. Next, we describe how these challenges facilitated the types of THCPs that were common in the migrants' data.

The most common structural challenges migrants encountered in the US healthcare system were: 1) documentation status; 2) US healthcare system complexity; and 3) affordability. These challenges are listed in order of the sequential difficulty they pose as barriers to health care. Documentation status directly shapes eligibility for health coverage in US health policy, in turn influencing the type of care migrants can access (Joseph, 2016, 2025; Park et al., 2024; Van Natta, 2023). Most non-citizens are excluded from various provisions in the 2010 Affordable Care Act (ACA). Even in states like California and Massachusetts that use their own funding to extend coverage to noncitizens, that coverage remains stratified by legal status (Joseph, 2025; Van Natta, 2023). Those with undocumented status had no/limited coverage options than those with green cards or naturalized citizenship. Carlos, an undocumented Dominican immigrant interviewed in 2013 shared: “Without papers, it is difficult to get a job, rent a house or apartment, anything ... You also can't get health insurance. They will treat you [in the hospital] but it's not equal.” Coverage options were further stratified regarding where it could be used: public coverage was not accepted in as many facilities as private coverage, which also mapped onto documentation status.

For immigrants who received coverage, the complexity of navigating the US healthcare system became another structural challenge shaping THCPs. Applying for and obtaining health coverage, figuring out how

that coverage works, and then finding a primary care physician are significant challenges for anyone needing care. Navigating the system also takes time, which means that immigrants do not receive timely care for health issues. As mentioned earlier, migrants with pre-migration healthcare exposure described longer wait times in the US as a reason for obtaining transnational care in their home countries.

Speaking a different language exacerbated structural challenges in navigating the system. Thirty-two of the 82 (40%) immigrants interviewed had limited English proficiency. Lack of patient-provider language concordance made it difficult to schedule appointments, understand English-only hospital signage, and communicate with providers during appointments and after hours. In such cases, patients needed medical interpreters to bridge those gaps, which meant longer wait times to see a provider. Dania, a Dominican interviewed in 2013, described how language hindered her communication: “It is difficult, I have cried so much when going to the hospital and they [staff] didn't know what I was saying. I couldn't even ask for anything to eat. And they treat us differently.” Finally, for those who effectively navigated the system to receive services, affordability posed another structural challenge given the exorbitant cost of care in the US (Cancarevic et al., 2021). Even those with health insurance routinely experience high prescription costs, co-pays, premiums, and out-of-pocket expenses (Cancarevic et al., 2021). Silvana, a Brazilian interviewed in 2015, explained she could not afford to get sick in the US:

The difference is that I never had to pay for health insurance in Brazil. And here, I joke that I can't afford to get sick. You work and you're in the middle class, but you can't get sick. It will break your budget.

Related to affordability, the fact that US health insurance does not cover dental or vision care often means additional costs for migrants. Thus, some forgo treatments they might have received in their home countries. Agustina, a Salvadoran interviewed in 2019 explained:

My [US public] insurance does not cover dental [care]. We have to pay and it is expensive. Sometimes, we lose our teeth because it is too expensive to go to the dentist. If I were in my country, I would go to the dentist.

Beyond the healthcare system, another structural challenge shaping migrants' US healthcare experiences includes the different services provided by pharmacies. In the US, pharmacists cannot examine or

prescribe medications to customers. But in Brazil and DR, people can consult pharmacists for minor ailments to get briefly examined and prescribed medication, a process that circumvents the need to see a doctor (Mena Lora et al., 2022; Rotta et al., 2023). Consequently, some immigrants described first going to a US pharmacy for similar help when they could not understand the US healthcare system or see a doctor. Adelia, a Brazilian quoted earlier, shared: “Here is so bureaucratic ... I don't go to the hospital. I go to the pharmacy and buy something, or I ask someone to send me medicine from Brazil that I'm used to taking.” Adelia circumvented complicated US healthcare bureaucracy by using over-the-counter medication she purchased at a US pharmacy or prescription medication sent to her from Brazil.

Socio-cultural challenges in the US healthcare system stemming from differences in the practice of medicine also facilitate migrants' THCPs (Levitt et al., 2023; Zhong et al., 2021). Language discordance presented structural challenges for patient-provider communication. But culturally, migrants described US providers' demeanor as distant, unfriendly, and “cold.” Some commented that US doctors spend more time on their computers than engaging with patients, which aligns with studies conducted in other contexts (Levitt et al., 2023; Shim, 2010). As described earlier, immigrants, especially those with positive pre-migration healthcare experiences, felt doctors in their countries made more eye contact and listened closely to patients.

Lack of cultural concordance also contributes to migrants' mistrust of and perceived discrimination from US providers. Victoria, a Dominican immigrant interviewed in 2012, recounted how her depression spiraled after substandard treatment from a White American psychiatrist:

She [doctor] thought I was using drugs because I was very hyper. I thought it was the medicine they prescribed me. I went into a really deep depression. If she had been Hispanic, she could have better explained to me what was happening.

Victoria's anecdote demonstrates how cultural and language differences translate to poor quality of care through misdiagnoses and ineffective treatments, sometimes with deadly consequences. At times, experiences like Victoria's made immigrants disengage from the formal healthcare system and seek remedies, alternative treatments, or care within their ethnic networks in the US and at home. Overall, these encounters reveal how numerous socio-cultural and structural challenges in the US healthcare system shape immigrants' engagement with THCPs.

2.4. Types of THCPs

Transnational networks were vital for migrants' transnational healthcare practices, especially amid myriad challenges navigating the US healthcare system. Immigrants remained in constant contact with family and acquaintances at home. Whenever health issues arose, immigrants explained them to acquaintances, who would connect with medical providers in the home country and either send medications through the mail or physically bring them during their own travels to the US. Analysis of immigrant interview data reveals that migrants engaged in three common types of THCPs. The first was receiving non-FDA approved medication from family or co-ethnic friends in the home country. Maria, a Brazilian interviewed in 2013, shared how receiving her anti-depressants was a lifesaver since she could not access them in the US:

I was getting anti-depressants from my [Brazilian] friend in Mexico ... Once it was by mail, but once she brought me when she came to visit me, or when her boss would come to Boston for business. I could not get the medication here.

One of the reasons Maria could not get medication was due to her undocumented status, which limited her access to affordable medication. Manuel, a Salvadoran interviewed in 2016 also observed: “A lot of people are self-medicating because they get meds from back home. Some of them managed to get some stuff from the internet.” Regardless of

documentation status and pre-migration healthcare experiences, Brazilian, Dominican, and Salvadoran respondents reported receiving and taking medication from their home countries.

Another THCP that immigrants engaged in was consulting with doctors abroad for health issues they were experiencing in the US. These consultations occurred over the computer using Skype, which was popular before Zoom during the pandemic. Mariana, an undocumented Brazilian immigrant interviewed in 2016, described a serious health condition she developed in the US. Feeling that her US doctor was inattentive, Mariana consulted with a Brazilian doctor online to get help:

I did consultations over the phone because I think our [Brazilian] doctors listen more, they're more attentive. I couldn't describe my symptoms but I had planned to leave and go to Brazil so I could die there. I found this doctor on the internet, a homeopathic doctor in São Paulo. He diagnosed what I had from a long distance.

Despite being undocumented, Mariana obtained the care she needed virtually. Like receiving medication from home, having online medical consultations was a THCP many migrants could engage in. While documentation status was not a hindrance, SES in migrants' transnational networks played a role. For virtual consultations, one needs reliable internet access and computer and health literacy, both of which are generally associated with middle-high SES. Moreover, some regions in migrants' home countries lack stable electricity or high-speed internet, demonstrating how differences in home-country SES and development might privilege some migrants over others. Of the three THCPs, this was the one in which fewer migrants engaged.

The final common THCP was traveling to the home country for treatment or surgeries, which aligns with the medical tourism literature (Bustamante, 2023; Connell, 2016). This was only an option for more structurally privileged immigrants with certain documentation statuses and financial means. Immigrants who were permanent residents or naturalized US citizens could receive procedures they thought were more affordable at home compared to the US. Others would get treatments at home because they felt more comfortable receiving care in those systems. Samantha, a Dominican immigrant interviewed in 2012, explained how Dominicans' challenges with US doctors shape their decision to get care at home. “When it comes to health, they look for the help they need, but many times they go back to the DR because there is no solution here. They go see a doctor there, they don't trust the system here.”

Alexandre, a Brazilian and naturalized US citizen interviewed in 2019, complained about US healthcare costs, which was a big factor in his decision to get most of his care in Brazil:

The price is so much [for US insurance]. You never know how much you will pay [for medical services]. ... I also have health insurance in Brazil because I can take a month of vacation. So, I pay here, and I pay there [for health insurance]. When I am there, I go to the doctors there ... My [private] insurance in Brazil is very good.

Both Samantha and Alexandre's quotes allude to the structural privilege of migrants who can go back home for care. While more Brazilians and some Dominicans mentioned going back home for care, this was related to their elevated pre-migration SES, documentation status, and SES in the US, which provided the mobility and financial resources to do so. Dominicans with green cards or US citizenship and limited pre-migration healthcare exposure as well as Salvadorans—most of whom had precarious statuses—were unable to travel for care. Thus, pre-migration healthcare exposure, documentation status, and SES in transnational networks significantly influenced the types of THCPs available to migrants.

2.5. Provider views on migrants' THCPs and their health consequences

Immigrants engaged in THCPs in response to structural and socio-cultural challenges they encountered in the US healthcare system. In

many cases, immigrants positively viewed THCPs because they helped them resolve healthcare issues that US providers could not address. Even though providers acknowledged their immigrant patients' engagement in THCPs, many expressed concerns about patients obtaining medications, treatments, and care outside the US. In this section, we utilize interview data from providers to demonstrate the THCPs they observe and their health consequences.

First, aligning with structural healthcare challenges that immigrants described, a few providers shared that their immigrant patients obtain endoscopies, cosmetic surgeries, and dental care abroad due to lower costs. Amelia, a BHC provider interviewed in 2013, explained the calculations her Latin American patients made when contemplating transnational care:

Like for example dental, they prefer to go to their country and get it. Certain types of surgery, they prefer to go to their country and do. So, we have a lot of patients who go to Brazil, do their dental treatment there, do their surgery there. We have patients, Latinos, who prefer to go to Colombia or El Salvador and do their treatment there. It's less expensive.

Providers accepted that THCPs were a reality among their patients. While most immigrants in this study reported engaging in them, providers observed that some nationalities engaged in them more often than others. Among the three nationalities of focus, providers described Brazilians as more frequently and actively using the three THCPs. Some providers also spoke about Brazilians' resourcefulness navigating the US and Brazilian healthcare systems compared to Dominican and Salvadoran immigrants. For example, Greg, a provider interviewed in 2013, shared how US-home country comparisons mapped onto nationality:

All of my immigrant patients will refer ... to their country of origin. There are Brazilian patients who will go back for periods sometimes as well. A number of Brazilian patients will continue ... to interact with the Brazilian medical system by continuing on prescriptions that they get from Brazil. Now as opposed to let's say many Central American patients, [they] will access medications from Central America here. Antibiotics, other things. Brazilian patients will also do that. They are able to get over-the-counter meds [there] which we would consider very controlled [require a prescription]. But they will also actually continue to receive prescriptions that someone may mail to them from a provider in Brazil.

While some felt Brazilians' THCP adeptness was related to nationality, others acknowledged that Brazilians' higher SES (pre-migration and in the US) likely facilitated their THCPs, especially when it came to going home for care. Grace, a provider interviewed in 2013 explained:

My Brazilian patients have very fine health literacy, much better than many American-born people, particularly the lower socioeconomic strata ... they [Brazilians] have much greater confidence in medical care in their home country ... they will do everything [there] and then come back ... But I have not really seen that with other group[s]. But Salvadoran patients seem much less developed [in terms of] networks and usually can't do that.

Despite noting these differences, providers cautioned all patients about THCPs due to potential health complications. Some of these complications arose from discontinuity of care during prolonged visits to home countries. This was especially problematic for patients requiring prescription medications or lab work to manage chronic health conditions while out of the country. Meredith, a provider interviewed in 2016, described her two Salvadoran patients' THCPs:

[They] go back to El Salvador at least six months a year ... they both have conditions that should be monitored at least every three to six months. Their kids are still here and are picking up their medicines and mailing them to El Salvador because they live in the middle of

nowhere ... there's no doctor, no access to care ... but we've been doing this for like three years.

Other providers mentioned health issues stemming from remedies or medications from abroad or post-surgery complications after patients returned to the US. Kevin, a provider interviewed in 2012, discouraged his patients from having surgeries abroad:

Some [patients] will go back for procedures because they'd rather do it under their Brazilian coverage or just want to be at home ... It's not uncommon, a patient shows up in our emergency room with an infection from a procedure they had in Brazil. We have no idea what the procedure was. I really discourage my patients from having surgery outside of the country and off the books.

Overall, though many immigrants viewed THCPs positively in terms of addressing unmet US medical needs, providers faced challenges integrating them into the care they provided. As providers suggested, having to juggle between the two can have serious medical consequences for both parties.

3. Discussion and conclusion

Immigrants engage in different types of transnational healthcare practices that range from taking medication from their home country to traveling home for medical services. Interviews with immigrants and providers in Boston reveal that immigrants' THCPs are a response to various structural and socio-cultural challenges they experience in the US healthcare system. We contextualize these findings below, highlighting a major contribution of the paper: the influence of pre-migration healthcare exposure on migrants' THCP orientation. Next, we discuss why THCPs are important in facilitating care and how they may contribute to medical complications. We conclude by describing how the evolving US socio-political climate may reconfigure immigrants' THCPs.

First, pre-migration healthcare exposure was crucially important for developing a THCP orientation. This finding is important as this is one of the first papers in the US context to empirically demonstrate how pre-migration healthcare context matters for navigating care transnationally. Because interviews collected data about immigrants' pre-migration education, health coverage, and care experiences, this provided valuable information about immigrants' socioeconomic status and health literacy.

Pre-migration SES establishes the foundation for migrants' transnational cultural health capital (Shim, 2010), which affects their healthcare navigation across borders. This is one of the first studies among migrants in the US to capture how those two characteristics influence THCPs. LaFleur and Romero (2018) indicate a similar impact of nationality and pre-migration social class among Columbian and Peruvian migrants in Brussels. In our article, having middle-or-high pre-migration SES or access to middle-and-high SES transnational networks eased migrants' engagement in THCPs. Migrants' pre-migration health insurance status and positive healthcare experiences further enabled travel back and forth for medical care at home compared to those with lower pre-migration SES.

Notably, pre-migration SES and access to the home healthcare system provide an important frame of reference for navigating healthcare systems. They were the foundations of a THCP orientation, which activated after people emigrated and encountered the US healthcare system. Migrants' US healthcare experiences engendered "here and there" comparisons that also activated transnational cultural health capital for the THCPs in which they would later engage.

Initial data analysis indicated that nationality differences may have shaped Brazilians' higher use of THCPs and providers' perceptions of their healthcare adeptness compared to Dominicans and Salvadorans. However, closer examination of migrants' pre-migration SES and healthcare exposure reveals that a middle-or-high SES pre-migration

background provides the health literacy, resources, and social networks that facilitate engagement with THCPs. Dominicans and Salvadorans, like Brazilians, with middle-or-high pre-migration SES backgrounds engaged in those THCPs that enabled them to navigate multiple systems or travel for care. Because SES differences shape capital, pre-migration SES can influence post-migration social positioning, further influencing THCP engagement. [Sun and Lin \(2025\)](#) observed a similar dynamic in their study on transnational caregiving among Chinese immigrants in the US. Migrants with green cards or naturalized US citizenship could more easily travel abroad for care, or when abroad for extended periods, have medications from the US sent to their home country. This reveals another layer of structural privilege unavailable to lower SES and undocumented migrants, regardless of nationality.

Structural and socio-cultural healthcare challenges in the US activated immigrants' THCP orientation and health capital, privileging those who had access to coverage and care before migration. However, even for migrants who did not have access to these resources, some THCPs were a direct and agentic response to healthcare challenges and unmet medical needs. Immigrants with lower SES or more precarious documentation status could still receive medication by mail or have telehealth consultations with providers in their home countries. THCPs also allowed immigrants to receive linguistically and culturally appropriate care that they struggled to access in the US. Building on previous studies describing medical tourism and migrants' THCPs in the US and US-Mexico border, this article suggests that challenges navigating the US healthcare system make THCPs a key source of care for immigrants outside the formal system ([Bustamante, 2023](#); [Connell, 2016](#); [Torres and Waldinger, 2015](#)).

Regardless of SES, legal status, and ethnic differences, THCPs are a work around for immigrants to address their medical needs. Through THCPs, immigrants assert their agency to fix problems not easily resolved in the US healthcare system. When Dominicans were sick and experienced delayed waiting times to see a provider with a medical interpreter, they took medication from the DR. If a medical procedure was too costly for Brazilians in the US, those with green cards or US citizenship and economic means returned home to have procedures done there. When Brazilians, Dominicans, and Salvadorans experienced cultural insensitivity from a provider, they scheduled a virtual appointment abroad. But as discussed, that agency develops in response to state-imposed immobility based on restrictive policies, legal status, or complex healthcare bureaucracies ([Sun and Lin, 2025](#)).

Although a response to healthcare challenges, THCPs can result in complications too and lead to misdiagnoses and higher costs for immigrants. This is a novel finding in studies of immigrants engaged in THCPs in the US. Studies of South Asian migrants in Hong Kong describe similar consequences in the Hong Kong healthcare system ([Leung and Ku, 2024](#); [Zhong et al., 2021](#)). In this study, some providers expressed concern when their patients showed up with complications from procedures done abroad. In some cases, US providers could not identify the problem to help these patients. Providers also described how non-FDA medications from abroad could have serious side effects, resulting in medical complications in the US. Thus, THCPs also pose potential risks for patients and create challenges for providers. They further reveal contrasting viewpoints on THCP efficacy between migrants and providers.

The limitations of this study are that its findings are derived from a small sample of immigrants and providers in Boston, a city where all residents were eligible for health coverage after comprehensive health reform. While these results are not generalizable, they shed light on structural and socio-cultural challenges immigrants face when trying to access care in the US. Collectively, these challenges shape their THCPs. Further qualitative and quantitative research is needed to explore how pre-migration socioeconomic status and healthcare exposure shape the types and extent of THCPs among migrants of other ethnicities across locales.

As the transnational movement of people continues in an increasingly anti-immigrant sociopolitical climate, discrimination towards

immigrants may facilitate deeper THCP engagement. In the US, increased enforcement puts everyone, including legal permanent residents and naturalized and US-born citizens, at risk of detention and deportation. THCPs like getting medication in the mail and transnational telehealth appointments may increase among immigrants of all statuses. THCPs involving travel could decrease amid difficulties re-entering the country and heightened scrutiny of those transporting medications ([Sun and Lin, 2025](#)). Global economic policies will similarly shape THCPs as increased US tariffs raise the costs of imported pharmaceuticals and medical equipment that people transport across borders. Thus, the nuances of THCPs will shift as migrants alter them in response to broader structural constraints. Future research should further explore THCPs to increase our understanding of the range of practices that exist, other barriers that contribute to immigrants' THCP engagement (e.g., by race and ethnicity), and complications that may arise from such practices, particularly amid declining social protections for both immigrants and citizens in the US and other host countries.

Ethics approval

The research data presented in the paper meets all necessary ethical standards as the first author obtained human subjects research approval from the Institutional Review Board (IRB) at Northeastern University.

Declaration of no use of generative AI and AI-assisted technologies in the writing process

During the preparation of this work, the authors used no generative AI at any point in the writing process and claim full responsibility for the content of the publication.

Funding

The first author received funding from the following agencies for the study: Ford Foundation Senior Fellowship, Fund for the Advancement of the Discipline supported by the American Sociological Association and National Science Foundation, Mellon Emerging Faculty Leaders Award from the Institute for Citizens and Scholars, and Robert Wood Johnson Foundation Scholars in Health Policy Program.

CRediT authorship contribution statement

Tiffany D. Joseph: Conceptualization, Formal analysis, Funding acquisition, Investigation, Methodology, Project administration, Resources, Supervision, Visualization, Writing – original draft, Writing – review & editing. **Vibhustuti Thapa:** Formal analysis, Resources, Visualization, Writing – original draft, Writing – review & editing.

Declaration of competing interest

The manuscript has not been published elsewhere and is not currently under review by any other journal. None of the authors have a conflict of interest to disclose.

Acknowledgements

The authors would like to thank Ken Chih-Yan Sun and external reviewers for their feedback on previous drafts of the manuscript.

Data availability

The authors do not have permission to share data.

References

- Ahmadinia, H., Eriksson-Backa, K., Nikou, S., 2022. Health-seeking behaviours of immigrants, asylum seekers and refugees in Europe: a systematic review of peer-reviewed articles. *J. Doc.* 78 (7), 18–41. <https://doi.org/10.1108/JD-10-2020-0168>.
- Amaro, E., Rodriguez, J., Jackson, D., Popovich, D., Frias, K.M., Castañeda, E., 2024. The impact of cultural health capital on market choice along the Texas-Mexico border. *J. Racial Ethn. Health Dispar.* 11 (2), 1139–1151. <https://doi.org/10.1007/s40615-023-01593-7>.
- Baldassar, L., 2014. Too sick to move: distant ‘crisis’ care in transnational families. *Int. Rev. Sociol.* 24 (3), 391–405. <https://doi.org/10.1080/03906701.2014.954328>.
- Becker, H., 1998. *Tricks of the Trade: How to Think About your Research While you are Doing it*. University of Chicago Press, Chicago.
- Bhattacharya, G., 2011. Global contexts, social capital, and acculturative stress: experiences of Indian immigrant men in New York City. *J. Immigr. Minority Health* 13 (4), 756–765. <https://doi.org/10.1007/s10903-011-9444-y>.
- Bryceson, D.F., 2019. Transnational families negotiating migration and care life cycles across nation-state borders. *J. Ethnic Migrat. Stud.* 45 (16), 3042–3064. <https://doi.org/10.1080/1369183X.2018.1547017>.
- Bustamante, A., 2019. US-Mexico cross-border health visitors: how Mexican border cities in the state of Baja California address unmet healthcare needs from US residents. *J. Ethnic Migrat. Stud.* 46 (20), 4230–4247. <https://doi.org/10.1080/1369183X.2019.1597473>.
- Bustamante, A., 2023. Medical tourism in the U.S.-Mexico border: California-Mexico cooperation after COVID-19. In: de Castro, R.F., Ganster, P., Gutierrez, C.G. (Eds.), *Calibaja: Emerging Stronger After COVID-19*. UC San Diego School of Global Policy and Strategy, pp. 83–92.
- Cancalevic, I., Plichtová, L., Haider Malik, B., 2021. Healthcare systems around the world. In: Tohid, H., Maibach, H. (Eds.), *International Medical Graduates in the United States*. Springer Nature Switzerland, pp. 45–80.
- Castañeda, H., Melo, M.A., 2014. Health care access for Latino mixed-status families: barriers, strategies, and implications for reform. *Am. Behav. Sci.* 58 (14), 1891–1909. <https://doi.org/10.1177/0002764214550290>.
- Chakrabarti, R., 2010. Therapeutic networks of pregnancy care: bengali immigrant women in New York City. *Soc. Sci. Med.* 71 (2), 362–369. <https://doi.org/10.1016/j.socscimed.2010.03.032>.
- Connell, J., 2016. Reducing the scale? From global images to border crossings in medical tourism. *Glob. Netw.* 16 (4), 531–550. <https://doi.org/10.1111/glob.12136>.
- Creswell, J., 2007. *Qualitative Inquire and Research Design: Choosing Among Five Approaches*, second ed. Sage Publications Ltd.
- Ellard-Gray, A., Jeffrey, N., Choubak, M., Crann, S., 2015. Finding the hidden participant: solutions for recruiting hidden, hard-to-reach, and vulnerable populations. *Int. J. Qual. Methods* 14 (5). <https://doi.org/10.1177/1609406915621420>.
- Escandell, X., Tapias, M., 2010. Transnational lives, travelling emotions and idioms of distress among Bolivian migrants in Spain. *J. Ethnic Migrat. Stud.* 36 (3), 407–423. <https://doi.org/10.1080/13691830903266093>.
- Faist, T., 2000. *The Volume and Dynamics of International Migration and Transnational Social Spaces*. Oxford University Press Inc.
- Grineski, S., 2011. Why parents cross for children's health care: transnational cultural capital in the United States–Mexico border region. *Soc. Theor. Health* 9, 256–274. <https://doi.org/10.1057/sth.2011.7>.
- Harding, J., 2013. *Qualitative Analysis from Start to Finish*. Sage Publications Ltd.
- Hoffman, B.R., 2025. *Borders of Care Immigrants, Migrants, and the Fight for Health Care in the United States*. University of Chicago Press.
- Horton, S., Cole, S., 2011. Medical returns: seeking health care in Mexico. *Soc. Sci. Med.* 72 (11), 1846–1852. <https://doi.org/10.1016/j.socscimed.2011.03.035>.
- Joseph, T., 2016. What healthcare reform means for immigrants: a comparison of the affordable care act and Massachusetts health reforms. *Journal of Health Policy, Politics, and Law* 41 (1), 101–116. <https://doi.org/10.1215/03616878-3445632>.
- Joseph, T., 2025. *Not all In: Race, Immigration, and Healthcare Exclusion in the Age of Obamacare*. Johns Hopkins University Press.
- Lafleur, J.-M., Mescoli, E., 2018. Creating undocumented EU migrants through welfare: a conceptualization of undeserving and precarious citizenship. *Sociology* 52 (3), 480–496. <https://doi.org/10.1177/003803851876>.
- Lafleur, J.-M., Romero, M.V., 2018. Combining transnational and intersectional approaches to immigrants' social protection: the case of Andean families' access to health. *CM* 6, 14. <https://doi.org/10.1186/s40878-018-0073-7>.
- Leung, S.K., Ku, H.B., 2024. Cross-border healthcare-seeking and utilization behaviours among ethnic minorities: exploring the nexus of the perceived better option and public health concerns. *BMC Public Health* 24, 1497. <https://doi.org/10.1186/s12889-024-18981-1>.
- Levitt, P., Dobbs, E., Sun, K.C.Y., Paul, R., 2023. *Transnational Social Protection: Social Welfare Across National Borders*. Oxford University Press.
- Madden, E.F., 2015. Cultural health capital on the margins: cultural resources for navigating healthcare in communities with limited access. *Soc. Sci. Med.* 133, 145–152. <https://doi.org/10.1016/j.socscimed.2015.04.006>.
- McFadden, A., Atkin, K., Renfrew, M.J., 2014. The impact of transnational migration on intergenerational transmission of knowledge and practice related to breast feeding. *Midwifery* 30 (4), 439–446. <https://doi.org/10.1016/j.midw.2013.04.012>.
- Mena Lora, A.J., Rojas-Fermin, R., Bisno, B., Almonte, M., Bleasdale, S.C., 2022. A nationwide survey of antimicrobial dispensation practices in pharmacies and bodegas in the Dominican Republic. *Antimicrobial Stewardship & Healthcare Epidemiology* 2 (1), e173. <https://doi.org/10.1017/ash.2022.314>.
- Menjívar, C., 2002. The ties that heal: guatemalan immigrant women's networks and medical treatment. *Int. Migrat. Rev.* 36 (2), 437–466. <http://www.jstor.org/stable/4149460>.
- Naderifar, M., Goli, H., Ghaljaie, F., 2017. Snowball sampling: a purposeful method of sampling in qualitative research. *Strides in Development of Medical Education* 14 (3). <https://doi.org/10.5812/sdme.67670>.
- Ormond, M., Lunt, N., 2020. Transnational medical travel: patient mobility, shifting health system entitlements and attachments. *J. Ethnic Migrat. Stud.* 46 (20), 4179–4192. <https://doi.org/10.1080/1369183X.2019.1597465>.
- Park, L.S.H., Hoekstra, E., Jimenez, A.M., 2024. *The Third Net: the Hidden System of Migrant Health Care*. New York University Press.
- Portes, A., Guarnizo, L.E., Landolt, P., 1999. The study of transnationalism: pitfalls and promise of an emergent research field. *Ethn. Racial Stud.* 22 (2), 217–237. <https://doi.org/10.1080/014198799329468>.
- Rotta, I., Lima, T., Tonin, F.S., 2023. Role of community pharmacy and pharmacists in self-care in Brazil. *Exploratory Research in Clinical and Social Pharmacy* 10, 100274. <https://doi.org/10.1016/j.rcsop.2023.100274>.
- Sanon, M.A., Spigner, C., McCullagh, M.C., 2016. Transnationalism and hypertension self-management among Haitian immigrants. *J. Transcult. Nurs.* 27 (2), 147–156. <https://doi.org/10.1177/1043659614543476>.
- Shim, J.K., 2010. Cultural health capital: a theoretical approach to understanding health care interactions and the dynamics of unequal treatment. *J. Health Soc. Behav.* 51 (1), 1–15. <https://doi.org/10.1177/0022146509361185>.
- Smith, R.C., Waisanen, D.J., Barbosa, G.Y., 2020. *Immigration and Strategic Public Health Communication: Lessons from the Transnational Seguro Popular Project*. Taylor and Francis.
- Strauss, A., Corbin, J., 1998. *Basics of Qualitative Research: Techniques and Procedures for Developing Grounded Theory*, third ed. Sage Publications.
- Sun, K.C., Lin, L., 2025. Confronting linked immobility: how Chinese migrants managed family crisis during a global catastrophe. *Soc. Forces*. <https://doi.org/10.1093/sf/soaf082>.
- Tiilikainen, M., Koehn, P.H., 2011. Transforming the boundaries of health care: insights from Somali migrants. *Med. Anthropol.* 30 (5), 518–544. <https://doi.org/10.1080/01459740.2011.577288>.
- Torres, J.M., Alcántara, C., Rudolph, K.E., Viruell-Fuentes, E.A., 2016. Cross-border ties as sources of risk and resilience. *J. Health Soc. Behav.* 54 (4), 436–452. <https://doi.org/10.1177/0022146516667534>.
- Torres, J.M., Casey, J.A., 2017. The centrality of social ties to climate migration and mental health. *BMC Public Health* 17, 600. <https://doi.org/10.1186/s12889-017-4508-0>.
- Torres, J.M., Waldinger, R., 2015. Civic stratification and the exclusion of undocumented immigrants from cross-border health care. *J. Health Soc. Behav.* 56 (4), 438–459. <https://doi.org/10.1177/0022146515610617>.
- Van Natta, M., 2023. *Medical Legal Violence: Health Care and Immigration Enforcement Against Latinx Noncitizens*. New York University Press.
- Vermot, C., 2015. Guilt: a gendered bond within the transnational family. *Emotion, Space, & Society* 16, 138–146. <https://doi.org/10.1016/j.emospa.2015.04.001>.
- Verschueren, H., 2020. The role and limits of European social security coordination in guaranteeing migrants social benefits. *Eur. J. Soc. Secur.* 22 (4), 390–402. <https://doi.org/10.1177/1388262720967938>.
- Vertovec, S., 2001. Transnationalism and identity. *J. Ethnic Migrat. Stud.* 27 (4), 573–582. <https://doi.org/10.1080/13691830120090386>.
- Virani, A., Wellstead, A.M., Howlett, M., 2020. The north-south policy divide in transnational healthcare: a comparative review of policy research on medical tourism in source and destination countries. *Glob. Health* 16 (1), 37. <https://doi.org/10.1186/s12992-020-00566-3>.
- Viruell-Fuentes, E.A., Schulz, A.J., 2009. Toward a dynamic conceptualization of social ties and context: implications for understanding immigrant and Latino health. *Am. J. Publ. Health* 99 (12), 2167–2175. <https://doi.org/10.2105/AJPH.2008.158956>.
- Zhong, L., Deng, B., Morrison, A.M., Coca-Stefaniak, J.A., Yang, L., 2021. Medical, health and wellness tourism research—A review of the literature (1970–2020) and research agenda. *Int. J. Environ. Res. Publ. Health* 18 (20), 10875. <https://doi.org/10.3390/ijerph182010875>.